VH

Vedant Multispeciality Hospital

Neurosciences · Cardiac care · Critical care

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discharge@vedanthospital.example

DISCHARGE SUMMARY

Patient Name	**: Mr. Suresh Babu Reddy**	IPD No.	: IP/26/08/0211
Age / Sex	: 67 Y / Male	Department	: Neurosciences
UHID	: GMS-10245	Admitted	: 02/08/2026 04:50 AM
Consultant	: Dr. Nandini Iyer, DM (Neurology)	Discharged	: 09/08/2026 01:00 PM

FINAL DIAGNOSIS

- **Acute ischaemic stroke, right MCA territory (NIHSS 7 on admission, 3 at discharge)**
- **Hypertension**
- **Type 2 diabetes mellitus**
- **Dyslipidaemia**

PRESENTING COMPLAINTS

Woke up with weakness of the left arm and leg and slurred speech; last seen well 9 hours earlier — outside the thrombolysis window.

COURSE IN HOSPITAL

Aspirin and clopidogrel (dual antiplatelet for 21 days) and atorvastatin 80 mg started. Swallow assessment passed on day 2. Physiotherapy from day 2; walking with a stick and one-person support at discharge. Echocardiography normal, no thrombus. 24-hour Holter: sinus rhythm, no atrial fibrillation.

KEY INVESTIGATIONS

LDL cholesterol	**142**	mg/dL	< 70
HbA1c	**7.8**	%	4.0 - 5.6
Serum Creatinine	**1.08**	mg/dL	0.70 - 1.30
Haemoglobin	**13.6**	g/dL	13.0 - 17.0

MEDICINES ON DISCHARGE

Medicine	Dose	When	Duration
1. Tab. Ecosprin 75 mg	1 tab	After lunch	21 days, then stop
2. Tab. Clopilet 75 mg	1 tab	After breakfast	Continue
3. Tab. Atorva 80 mg	1 tab	At night	Continue
4. Tab. Telma 40 mg	1 tab	Morning	Continue
5. Tab. Glycomet 500 mg	1 tab	Twice daily	Continue

ADVICE ON DISCHARGE

- Daily physiotherapy as taught
- BP target below 130/80; sugar monitoring
- Report new weakness, speech or vision problems immediately — call 108

FOLLOW-UP

Neurology OPD after 3 weeks with lipid profile.

Dr. Nandini Iyer
DM (Neurology), Consultant Neurologist